

CONSULTATION & COLLABORATION

Case Consultation Presentation Template

A de-identified one-page format that turns “I’m stuck” into a sharp consultation question, with a de-identification checklist and a decision record.

AUDIENCE

Psychologists · Psychiatrists

FORMAT

Form + checklist

VERSION

1.0 · Sept 2026

TIME TO COMPLETE

10 minutes per case

WHAT IT IS

A structured form for presenting a case to colleagues without identifying the patient, and a response sheet for recording what you learned and what you decided.

USE IT WHEN

You are bringing a case to a peer group, a supervisor, or a one-off consultation with a colleague.

CUSTOMIZE FIRST

Your group's time limit, the type of consultation you most often need, and your own patient-ID convention.

Start with the question

Consultations go better when the presenter arrives with a question rather than a story. A useful question is specific, answerable in the time available, and tells colleagues what kind of help you want. Choose the type that fits, then write the question in one sentence.

Type of consultation	Example of a good question
Diagnostic clarification	What else should I be ruling out given the pattern of onset and family history?
Treatment impasse	We have plateaued after ten sessions. What am I not seeing about the relationship or the model?
Risk	I moved from weekly to twice-weekly sessions and a safety plan. Is my level of care decision reasonable?
Ethics / legal	I am unsure whether this situation triggers a duty to report. What is my reasoning missing?
Boundaries / countertransference	I notice I dread this patient's sessions. What might that be telling me?
Medication (psychiatrists and split treatment)	The patient has partial response at an adequate dose. What would you consider next?
Termination / transfer	How do I approach ending with a patient who depends on me during a crisis?

The de-identification checklist

Complete before you present. HIPAA's Safe Harbor method lists eighteen kinds of identifiers that make information "identified." You are not formally bound by Safe Harbor when consulting for treatment, but it is a reliable floor.

- ☐ No names, initials, nicknames or usernames.
- ☐ No geographic detail smaller than a state (city, neighborhood, school, workplace, place of worship).
- ☐ No dates related to the person except year (no birth date, admission, session or event dates). Use ages, and for older adults over 89 use "older adult."
- ☐ No phone, fax, email, address, URLs, IP addresses, account, license, vehicle or device numbers.
- ☐ No photographs, recordings, or unique codes that link back to the person.
- ☐ No details that are individually unique: an unusual profession, a famous relative, a rare event that made the news.
- ☐ The "stranger and neighbor" test: a stranger could not recognize this person, and neither could someone in the same town.
- ☐ Small-community caution: where the community, cultural group or specialty is small, remove or generalize even more, or seek consent.

WHEN DE-IDENTIFICATION IS NOT ENOUGH

Some cases are recognizable because of their story. If you cannot present a case without making it recognizable, obtain the patient's written consent, choose a different consultant outside the patient's community, or present the question in the abstract.

Case consultation form (one page)

Date

Consultation group / consultant

Patient ID (your code)

Type of consultation

Time requested

Decision needed by

My consultation question

One sentence

De-identified snapshot

Age range, gender identity (if relevant), cultural and social context

Presenting concern and why now

Relevant history (development, trauma, medical, psychiatric, family, substance use)

Working diagnosis / formulation

Treatment so far

Modality and frequency

Duration

Setting (in person, telehealth)

Measures and trend

Medication (name, dose, response) and prescriber

Risk summary

Suicide / self-harm / violence: current and past

Protective factors and access to means

Current level of care and why

The relationship and my reaction

Alliance and any ruptures

What I feel in the room and what I notice about my responses

What I have tried and where I am stuck

Interventions tried and their effect

What I am stuck on, worried about, or avoiding

Constraints (time, payer, family, legal, geography)

Ethics decision steps (use when the question is “What should I do?”)

1. Name the issue in one sentence, and the people affected.
2. Identify the relevant ethics code standards, law and licensing-board rules, and any organizational policies.
3. List the options, including doing nothing and the most cautious option.
4. For each option: who benefits, who may be harmed, and what the worst realistic outcome is.
5. Consult: a colleague, your professional association’s ethics service, your liability carrier’s risk-management line, or an attorney when law is involved.
6. Decide, and record your reasoning.
7. Review the outcome and what you would do differently.

Consultation response sheet

Consultation date and with whom

Main perspectives offered

Views that differed from mine, and why

What I decided and why

Action items and date to review

Documented in log (date)

IN PSYALLIANCE

Town Hall specialism channels are for questions in the abstract. Use this form for case-level consultation in a private group or one-to-one. Never post case detail publicly.

Sources and further reading

- APA Ethics Code, Standard 4.06 Consultations (apa.org/ethics/code).
- HHS, Guidance regarding methods for de-identification of protected health information: Safe Harbor method (hhs.gov/hipaa).
- APA Ethics Committee and state psychological association ethics consultation services.

ABOUT THIS DOCUMENT

Prepared by PsyAlliance from publicly available professional and regulatory sources, reviewed against them as of September 2026. It is an educational template and practice aid. It is not legal, ethical, clinical or billing advice, and it does not create a professional relationship. Laws, licensing-board rules and payer requirements vary by state and change over time. Verify against your licensing board, professional association, payer contracts and, where the stakes warrant it, a qualified attorney before relying on it.

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